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Correct
Answer

Next Q

Practice Exam - 1

Question 101 of 117



A 46-year-old man returns for his annual transplant evaluation. He received a heart transplant 5 years ago for an inherited cardiomyopathy. Except for an episode of ISHLT 2R rejection in the first year, he has had an unremarkable course. He feels well overall, but becomes short of breath walking more than 2 blocks. Medications: tacrolimus 2 mg bid, mycophenolate mofetil 1,000 mg bid, prednisone 2.5 mg, rosuvastatin 10 mg, aspirin 81 mg, vitamin D 1,000 U and amlodipine 2.5 mg. On exam he is normotensive and otherwise unremarkable. EKG shows sinus rhythm with a right bundle branch block. Echocardiogram shows an LVEF 40%, mild RV dysfunction and mild tricuspid regurgitation. Coronary angiography shows some distal pruning with an 80% mid-LAD lesion.

In addition to starting everolimus, which of the following is the best next step in his management?

- A. Increase rosuvastatin
-  B. Stop prednisone
- C. Perform percutaneous coronary intervention
-  D. Refer for repeat heart transplantation
- E. Switch cyclosporine for tacrolimus

Commentary

Testing Point: The Board-certified AHFTC specialist understands the treatment algorithm for allograft vasculopathy including when to refer for re-transplantation.

Correct Answer: D. Refer for repeat heart transplantation

